Fax

To:	pam hht	From:	dr weber
Fax:	3033250150	Fax:	
Company:		Voice:	

Date: March 3, 2026
Subject:

Comments:

The medical information in this FAX message is confidential and protected by both State and Federal Law.
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If you have received this FAX in error, please notify us immediately at the telephone number indicated above
and either destroy these documents or return the originals to us by mail. Thank you.

Fax

To: WEBER, IAN	**From:** Pam Health at Home - Denver
Fax: 17206274981	**Pages:** 7
Phone:	**Date:** 03-03-2026 4:08 PM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please sign, date, and return all pages to 1-303-325-0150.
Thank you!

PAM Health at Home - Denver
9025 E. Kenyon Ave, Suite 100, Denver, CO 80237
Ph: 720-808-0001; Fax: 303-325-0150

IMPORTANT - PLEASE NOTE: The information transmitted herein is intended only for the person or entity to which it is addressed and may contain confidential, proprietary, protected, and/or privileged material. Any review, retransmission, disclosure, dissemination or other use of this information by persons or entities other than the intended recipient is prohibited. If you received this in error please contact the sender and destroy this facsimile.

Order No:485-25864

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
L586103	02/18/2026	02/18/2026 - 04/18/2026	9607	

6. Patient's Name and Address
MONTEZ, LAURA M (9607)
243 W 80TH AVE UNIT 5106
Denver, CO 80221
Phone: (720) 338-1651

7. Provider's Name, Address and Telephone Number
PAM Health at Home
9025 East Kenyon Ave Ste 50
Denver, CO 80237-1980
Phone: (720) 808-0001, Fax: (720) 279-4371

8. Date of Birth: 06/18/1971 **9. Sex:** ☐ M ☑ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Adult Low Dose Aspirin 81 mg tablet,delayed release [Amount Of/ Dosage Form: 1 Tablet]	81mg	2 times a day	oral	LS
Amitriptyline 25 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	25mg	bed	oral	LS
Ascorbic acid (vitamin C) 500 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	500	daily	oral	LS
Calcium 600 with Vitamin D3 600 mg-10 mcg (400 unit) chewable tablet [Amount Of/ Dosage Form: 1 Tablet]	600	daily	oral	LS
Cholecalciferol (vitamin D3) 25 mcg (1,000 unit) capsule [Amount Of/ Dosage Form: 1 Capsule]	1000	daily	oral	LS
Hydromorphone 2 mg tablet [Amount Of/ Dosage Form: 2 Tablet]	2	every 4-6 hours	oral	LS - PRN/As Needed
Iron (ferrous sulfate) 325 mg (65 mg iron) tablet [Amount Of/ Dosage Form: 1 Tablet]	325mg	daily	oral	LS
Klayesta 100,000 unit/gram topical powder [Amount Of/ Dosage Form: 1 powder]	100000	daily	topical	LS - PRN/As Needed
Miralax 17 gram oral powder packet [Amount Of/ Dosage Form: 1 Packet]	17g	daily	oral	LS
Pantoprazole 40 mg tablet,delayed release [Amount Of/ Dosage Form: 1 Tablet]	40mg	daily	oral	LS
Senna Lax 8.6 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	8.6mg	2 times a day	oral	LS - PRN/As Needed
Tylenol Extra Strength 500 mg tablet [Amount Of/ Dosage Form: 2 Tablet]	500	every 6 hours	oral	LS - PRN/As Needed
VitaJoy Biotin 2,500 mcg chewable tablet [Amount Of/ Dosage Form: 1 Tablet]	2500	daily	oral	LS
Zepbound 5 mg/0.5 mL subcutaneous pen injector [Amount Of/ Dosage Form: 1 pen injector]	5mg	weekly	subcutaneous	LS
Zinc sulfate 25 mg zinc (110 mg) tablet [Amount Of/ Dosage Form: 1 Tablet]	25	daily	oral	LS

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11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
Z47.1	Aftercare following joint replacement surgery		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
Z96.641	Presence of right artificial hip joint		
I10	Essential (primary) hypertension		
G47.33	Obstructive sleep apnea (adult) (pediatric)		
M19.90	Unspecified osteoarthritis, unspecified site		
Z79.82	Long term (current) use of aspirin		
Z79.85	Long-term (current) use of injectable non-insulin antidiabetic drugs		
Z79.891	Long term (current) use of opiate analgesic		

14. DME and Supplies:
DME: Cane, Elevated Toilet Seat, Tub/Shower Bench, Walker
ALCOHOL WIPES, NS GLOVES, PROBE COVERS, PPE MASK

15. Safety Measures:
911 Emergency / Protocol, Assistance with IADLs, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Keep Pathways Clear, Medication Safety / Storage, RIGHT HIP PRECAUTIONS, Safe in Movements, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
No Added Salt,
HEART HEALTHY

17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☑ 9- Cane

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☑ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☑ D- Other (Specify):SBA

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☑ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:

Frequency
PT Frequency: 1W1, 2W2 Starting Week of 02/18/2026,DC OT ORDER

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Pain: > 7

PT

PHYSICAL THERAPY:
• Evaluation and Treatment: eval/rx strength,endurance,balance and safety training

Assessment Summary
patient under went a R tha 2/2 OA without complications. patient is very pleasant and lives in a small apartment with 2 cats. 6 stairs to enter/leave apartment with handles. taught patient importance of adhering to posterior hip precautions during all mobility tasks. pts mother is able and willing to assist pt in adls/mobility

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.

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Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. LAURA MONTEZ is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred BY HANNAH CAROL AUDET, PA-C on 02.10.2026 and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 02.18.2026.

Risk of Hospitalization

Risks: Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications; Currently reports exhaustion;
Interventions: Referrals - Dietary Consultant Other: na; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); More than 2 Secondary Diagnoses (M1022 & M1024); Lives Alone (M1100) ; ADL Assistance Needed (M2102 & M2110) ; Home Safety Risks

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directive

Patient/Caregiver reports no Advanced Directives in place

Homebound Status

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Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Dependent upon adaptive device(s);
- Patient Chronic and Stable: No
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

PT

HEP: - LTG

• LTG-Patient and/or caregiver will complete Home Exercise Program with _no____(assist) within ___3___weeks. or at end of episode.

HEP: - STG

• STG-Patient and/or caregiver will complete Home Exercise Program with ____cg_(assist) within __1____weeks.

MOBILITY/GAIT: - LTG

• LTG-Patient will demonstrate improved mobility as evidenced by the increased ability to ambulate with _____cane/4ww_____(device) for ___150ft___(distance) with ___mi____(assist) in order to ___increase endurance_____(activity) within __3___weeks. or at end of episode.

MOBILITY/GAIT: - STG

• STG-Patient will demonstrate improved mobility as evidenced by the increased ability to ambulate with ____4ww_____(device) for ___90ft___(distance) with ___sba____(assist) in order to _____increase endurance_____(activity) within 1_____weeks.

TRANSFER TRAINING: - LTG

• LTG-The patient will improve _sit to stand_____ transfer from __mod_____(assist) to ___no____(assist) using ____cane/4ww_____ (assistive device) in order to be able to increase strength_____(activity) within ___3__weeks. or at end of episode.

TRANSFER TRAINING: - STG

• STG-The patient will improve __sit to stand_____ transfer from __mod_____(assist) to ____cg___(assist) using _____4ww/fww____ (assistive device) in order to be able to _____increase strength_____(activity) within ____1_weeks.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)	25. Date HHA Received Signed

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Verbal SOC Date: 02/18/2026
Signed By: Coleman, Kerry - PT on 02/18/2026 01:34 PM, Cosigned by Strait, Donna - RN

POT:

24. Physician's Name and Address
IAN WEBER MD
500 W 144TH AVE STE 120
WESTMINSTER, CO 80023,NPI: 1205922655, License#: 55805,Phone: (720) 627-4980, Fax: (720) 627-4981

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:

3/3/26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.